CONSULT FOR AFRICA

The Marketing Handbook

Consult for Africa, CadreHealth, Maarova and Mezo Health

Prepared for Ejiro, Marketing

Consult for Africa internal. Not for circulation outside the firm.

17 September 2026

hello@consultforafrica.com / +234 913 813 8553 / consultforafrica.com

Lagos and Abuja, Nigeria

A letter from Debo

Ejiro,

Welcome. I want to start by telling you what you have actually walked into, because the shape of this firm is not obvious from the website and you will waste weeks if you have to work it out by inference.

Consult for Africa is a healthcare management consultancy, but we are not the kind that writes a report and leaves. We are hired by hospital owners, group chief executives, family offices and boards who are usually in one of three positions. They have built something that is not working as well as they expected and they cannot see why. They are about to commit serious capital and want somebody who has run a hospital to tell them the truth before they sign. Or they need somebody to actually run the thing, in which case we take the operational reins rather than advise from a distance. Most of our work starts as the first and turns into the second or third, which is why almost every engagement you will read about in this handbook grew after it started.

Around that core we have built products, and this is the part people get wrong when they talk about us. Maarova, CadreHealth and Mezo Health are not side projects and they are not a software business bolted onto a consultancy. They exist because the same three things break in every African health institution we are called into, which are leadership, workforce and the economics of a practice, and we got tired of recommending fixes that nobody could buy anywhere. So we built them. That means the products carry the firm's credibility and the firm carries the products' proof, and your job is to make that relationship visible without making it sound like we are selling software.

A few things I want to say plainly up front about how we communicate.

The first is that our clients are senior people, most of them clinicians, and several of them know me personally. That sets the register for everything. We write to people, not about them. We do not perform enthusiasm, we do not use exclamation marks, and we never write a sentence that a consultant reading it on a phone between cases would find embarrassing.

The second is that our work is confidential and a written document travels. I am strict about this because I have seen what happens when a consultant's paper ends up somewhere it was never meant to go. In this handbook you will find the real engagements, the real numbers and the real findings, because you cannot market work you do not understand. What goes out into the world is the method and the lesson, never the client, and Part Four sets out exactly where that line sits for each piece of work.

The third is that our proof is unusually good and we have been bad at using it. We have traced ten surgical cases end to end through a private clinic to find where the money leaked. We have recovered a building's true geometry out of a CAD file and found that a client's brief was wrong by two hundred square metres. We have told a board that their revenue record was a nursing handover sheet. That is the material. Most firms in this market are publishing generic thought leadership because they have nothing like it, so the opportunity in front of you is not to invent a voice for us. It is to show the work.

I do not expect you to absorb all of this at once. Read Part One and Part Two this week, read Part Three before you write anything, and treat Part Four as the library you go back to. When you are unsure whether something can be said, ask me rather than softening it, because a softened version of a sharp finding is usually worse than either the finding or silence.

The natural first step is for you and I to sit down after you have read this and go through the three campaigns I want running before the end of the year.

Warm regards,

Dr Debo Odulana

Founding Partner, Consult for Africa

How to use this handbook

This is one document covering four brands, and it is long because the alternative was four documents that would drift apart. Read it in this order.

Part	What it covers	When you need it
1	The firm: what we sell, how we work, how we price, who we are	First week. Read once, properly.
2	The four brands: CFA, CadreHealth, Maarova, Mezo Health	First week, alongside Part One.
3	Positioning, targeting, channels, voice and the rules	Before you write anything at all.
4	Nine anonymised case studies with the methodology shown	Your working library. Return to it constantly.
5	The method library: the reusable things we do, named	When you need an angle and have run out.
6	Your first ninety days	Week one, then again at week four.
7	Appendices: glossary, boilerplate, clearance checklist	Whenever you are unsure.

Three conventions used throughout. **Internal only** marks material that must never leave the firm in any form. **Publishable** marks material already cleared to go out. **Clear with Debo** means the substance is usable but the specific framing needs his sign off before it is published.

Part One: The firm

1.1 What Consult for Africa is

Consult for Africa is a healthcare management consulting firm founded in 2024, operating from Lagos and Abuja, working across Nigeria and into the wider continent. It is written **Consult for Africa** in body copy and **CFA** in short form. You will also see **C4A** in older material and in file names. Both are us. Prefer CFA in anything new, and spell the name in full on first use in any external piece.

The legal billing entity is **Consult for Africa Management Services Limited**. That name appears on invoices and contracts. It does not appear in marketing copy.

The one line version, which is on the brand guide and is the one to use:

> Consult for Africa helps hospitals, health systems, investors and governments turn ambition into delivery. Built by clinicians and operators who know the system from the inside, we do not hand you a report and walk away. We build, staff, and stand behind the outcome.

The positioning statement behind it:

> Consult for Africa is a healthcare management consulting firm. We turn around struggling hospitals, install clinical governance, and stand up fractional leadership for African health systems.

Tagline on the brand guide: **Healthcare performance, restored.**

1.2 The thesis the firm is built on

This matters more than any tagline, because it is the argument you are actually making in every piece of content.

Across the continent, the gap between a struggling hospital and a thriving one is rarely the building or the equipment. It is execution. It is leadership. It is people.

That is not a slogan we reverse engineered. It is what nine engagements have found, repeatedly and independently. A paediatric facility that looked like it had a stock control problem had a culture and incentives problem. A surgeon who believed his growth was constrained by his clinic's postcode was actually constrained by a referral pipeline he had never built. A group converting a building found that the brief it had been working to was wrong about the building. In every case the presenting complaint was physical or financial, and the binding constraint was organisational.

Use this. It is the spine of our thought leadership and it is true.

1.3 The seven service lines

These are the published services with their own pages on consultforafrica.com.

Service line	What it actually means	Typical buyer
Clinical Governance	Safety culture, standards of work, clinical audit, incident systems, quality accreditation readiness	Medical director, board quality committee
Strategy & Growth	Market entry, growth strategy, service line design, pricing and tariff, feasibility, financial modelling	Owner, group CEO, board
Fractional Leadership	Senior operators placed part time into a client: interim COO, medical director, commercial lead	Owner who cannot yet afford or find a full time hire
Digital Health	System selection, EMR and HIS advisory, digitisation programmes, product strategy	CEO, CIO, hospital group
Health Systems Strengthening	Programme design and delivery for payers, ministries, federations and donors	Government, HMO, development finance
Diaspora Expertise	Access to senior Nigerian clinicians abroad for advisory, teaching, case review and episodic clinical work	Hospital needing scarce specialist capability
Healthcare Marketing Agency	Positioning, brand, digital and patient acquisition for healthcare providers	Clinic, hospital, specialist practice

Alongside the published seven, the work we are most often actually bought for, and which you should treat as real service lines in content even though they sit under the headings above:

- **The organisational audit.** A structured diagnostic across ten domains of a facility, delivered as a report with a formal maturity assessment and a set of decisions for the board.
- **Turnaround and growth mandates.** A target, a set of levers, and delivery against it.
- **Board governance.** Setting up boards, committees, delegation of authority, director training.
- **Venture structuring.** Holding companies, joint ventures, cap tables, shareholder agreements, working alongside the client's counsel.
- **Build and operate.** We design the facility or the venture, set it up, then manage it under a management agreement.

1.4 How the firm actually works

There is a progression that recurs, and it is worth understanding because it shapes what content is useful at each stage.

Stage one, the options note. Before a prospect has engaged us, they get a short note that lays out the strategic options, gives capital and revenue as ranges rather than point estimates, states what has to be true and what kills each option, and is explicit about what we cannot yet know. It is deliberately not a full feasibility study. A confident twenty six page plan answers every question and leaves nobody a reason to call.

Stage two, the diagnostic. We go in and find out what is actually there. This is where our methods live and where almost all of our best content comes from.

Stage three, the strategy or the plan. Built on the diagnostic, presented as decisions with options rather than recommendations, so the board is choosing rather than being told.

Stage four, delivery. We execute, usually with a named team and a milestone schedule.

Stage five, operate. On some mandates we then run it, under a management agreement with an operator fee.

Not every engagement goes all the way. Several have. The content implication is that our case studies are rarely "we advised and left", and that is our single biggest differentiator against the international firms and the local boutiques alike.

1.5 How we price, and why it matters to marketing

You will not write about fees. You need to understand them anyway, because the pricing philosophy is a positioning statement and it leaks into everything.

Resourced day rates, published. For setup and delivery programmes we price grades by days by rate and show the build. Partner 750k, principal 450k, senior consultant 300k, consultant 190k, analyst 120k, per day. The argument is that a transparent day rate build survives a scope change and keeps the negotiation off percentages of the client's capital.

Standard, then one visible discount, then net. Our proposals show the full standard fee, a single named concession line, and the net. Full transparency about what has been given up. This is deliberate and it works.

Anchor, do not open at the floor. We publish an opening position and hold concessions back for the negotiation. We never volunteer a cap, never benchmark ourselves at the bottom of a comparator range in a client document, and never defend a price before anybody has questioned it.

Build and operate has five separate bases, because it is five different activities: delivery, capital efficiency, establishment, management, equity. The finding worth knowing is that a percentage of capital development fee is the wrong instrument whenever the client wants lean capital, because it punishes us for doing exactly what was asked. We replaced it with a fixed delivery fee plus a share of the capital saved.

What this means for you. Never put a price in marketing copy. Never justify a price that nobody has challenged. If a piece of content invites a fee conversation, that is the correct outcome, and the conversation happens on a call.

Internal only. None of the rates in this section appear in any external material.

1.6 The people

Dr Debo Odulana, Founding Partner. Formally Dr Jubril Adebowale Odulana. MBBS, University of Sharjah. MSc International Health Management, Imperial College Business School. Former Chief Executive of Cedarcrest Hospitals, Abuja. Former Chief Innovation and Strategy Officer at Evercare Hospital Lekki.

Founder of Doctoora, built to twenty one states and sold to Evercare. President of the Doctors Foundation for Care, a diaspora physician network of more than five hundred doctors. He is also founder and chief executive of Covally.

He is a doctor and is referred to as **Dr Debo Odulana** in any formal context, never "Debo Odulana". In informal social copy, "Debo" is fine.

Tito Ipinmoye, Director of Product and Strategy. Clinician and healthcare executive, also Chief Product Officer at Cooked Indoors, Executive MBA at ESADE Barcelona. She leads day to day execution on audit engagements.

Dorothy, Director of Business Development and Growth. Owns the BD practice and the payer and corporate relationships.

Supporting bench of analysts and consultants drawn from our own talent pipeline, plus the diaspora network for specialist input.

1.7 Contact and channels of record

- Web: consultforafrica.com
- Email: hello@consultforafrica.com and partnerships@consultforafrica.com
- Phone and WhatsApp: +234 913 813 8553
- Offices: Lagos and Abuja, Nigeria
- LinkedIn: Consult For Africa
- X: @consultforafrica
- Instagram: @consultforafrica

hello@ is the sender of record for everything we send. Campaign email goes out from that address and not from a platform address, because that is the mailbox Debo checks and an outbound campaign that leaves no trace in it is invisible to the firm.

1.8 The wider stable

Beyond the four brands this handbook covers in depth, the firm holds or has built a number of ventures. You should know they exist. Most are **not** yet things you may write about publicly.

Venture	What it is	Marketing status
Cooked Indoors	Medical nutrition and clinically informed catering, Lagos. Live at cookedindoors.com	Publishable. Named in our own advertising already.
Covally	Healthcare cooperative for hospital staff, founded by Debo	Internal until launch
Connexxum	B2B healthcare procurement, pooled buying, escrow	Internal until beta
Cureva	Medical tourism marketplace with a patient advisor	Internal until pilot
HospitlOS	Hospital operating system used inside managed facilities	Internal. Never named in client copy.

Venture	What it is	Marketing status
Ortho Surplus	Orthopaedic implant supply for West Africa, orthosurplus.com	Thin public presence. Clear anything with Debo.
ilé	Residential eldercare and home care venture, in build	Own brand, deliberately carries no CFA identity. Do not associate them publicly.
Maarova Vet	Executive integrity vetting. Parked pending research	Do not mention at all.

The rule of thumb: if it does not have a live public page you can point at, it is not ready to be marketed.

Part Two: The four brands

2.1 Brand architecture

There is one firm and three products, and the relationship between them is specific.

Consult for Africa is the parent and the credibility source. It is the brand that gets into the boardroom. Everything else inherits trust from it.

Maarova, CadreHealth and Mezo Health are products of the firm. They live under CFA today, on CFA routes and CFA infrastructure, and each is built to be able to leave and stand alone later. Maarova sits at consultforafrica.com/maarova. CadreHealth sits at consultforafrica.com/oncadre with oncadre.com as its intended home. Mezo already runs on its own codebase, its own database and its own domain at mezohealth.com.

The three products do three different jobs for three different buyers, and confusing them is the single most common mistake in our own past copy.

	Maarova	CadreHealth	Mezo Health
Sells to	Institutions	Individual professionals	Individual doctors, and facilities
The job	Know and grow your leaders	Own your career	Run a private practice without building one
Money comes from	The hospital or group	The professional, and employers	Subscription plus practice economics
Voice	Calm, evidence led	Peer to peer	Aspirational, respectful of seniority
Primary channel	LinkedIn and direct	X, Instagram, WhatsApp	Direct and referral

The flywheel, which is the story worth telling internally and never told as a sales pitch externally. CadreHealth builds the professional pool and produces workforce data. That data is CFA's thought leadership and it is the sourcing engine for CFA's recruitment and fractional placements. CFA's engagements surface leadership problems, which is what Maarova measures and fixes. Doctors inside CadreHealth who want a private practice rather than a job are the population Mezo exists for. Facilities CFA manages are places Mezo doctors can practise.

2.2 Consult for Africa

Audience. Hospital owners and promoters, group chief executives, medical directors, boards, family offices, health system executives, development financiers, ministries. Nigeria first, then Ghana, Kenya and the wider continent.

Palette. Brand Navy `#0B3C5D`. Brand Gold `#D4AF37`. Deep Navy `#081521` for dark surfaces. Teal `#1F7A8C` as a secondary accent for charts and data. Body text `#1F2937`, muted `#6B7280`, surface `#F8FAFC`.

Typography. Currently the system stack. A neutral grotesque for the wordmark and a transitional serif for long form are recommended and unresolved. If you commission a designer, this is an open brief.

Logo. Wordmark in use, PNG only. There is no SVG master and that is an open item. Two assets you will need: `docs/c4a-logo.png` has a baked white box and is wrong on navy. Use `docs/c4a-logo-reversed.png`, the white knockout with a transparent background, on dark backgrounds.

Voice principles.

- **Senior, not salesy.** Read like a strategy partner who has run a hospital, not a software marketer. Gravitas earns the meeting.
- **Operator vocabulary.** Use the words clients use: turnaround, governance, capex, occupancy, locum, secondment, tariff, receivables, establishment.
- **Africa specific proof.** Lead with continent relevant numbers and named markets. Generic global benchmarks land flat.

Say: hospital turnaround and clinical governance; fractional leadership for African health systems; operations in Nigeria, Ghana, Kenya and beyond.

Avoid: disruptive, synergy, revolutionary; AI powered consulting; borrowed Silicon Valley voice. We are an Africa firm.

Sample headlines from the brand guide. Hero: "Healthcare performance transformation across Africa." Sub: "Hospital turnaround, clinical governance, strategy, and operational excellence." Proof: "Operations in Nigeria, Ghana, Kenya, and across the continent." Call to action: "Book a discovery call."

The cleared proof point. Consult for Africa is the management partner for Pastor Dr Tony Rapu's twenty one bed drug and addiction rehabilitation centre in Lekki, Lagos. This has already run in our own advertising, so it is publishable as written. It is a strong line because it proves the operate half of the model, not just the advise half.

2.3 CadreHealth

What it is. A career intelligence and workforce platform for Nigerian healthcare professionals. It is deliberately not a job board. It is a professional identity and intelligence layer that helps a healthcare worker earn more, prove their credentials and steer their own career.

Tagline. Your career. Your call.

Positioning statement. CadreHealth is the career platform for Nigerian healthcare professionals. Real salary data, honest hospital reviews, verified credentials, and a readiness score calibrated to UK, US, Canada and Gulf markets.

Audience. All sixteen healthcare cadres, not just doctors. Medicine, dentistry, nursing, midwifery, pharmacy, medical laboratory science, radiography and imaging, rehabilitation therapy, optometry, community health, environmental health, nutrition and dietetics, psychology and social work, public health,

health administration, biomedical engineering. Core age band is roughly twenty five to forty five, early to mid career. Secondary audience is the diaspora. Tertiary is medical students choosing where to do housemanship, who have the highest search intent of any group.

The audience insight that drives everything. Around eighty per cent of Nigerian healthcare workers intend to emigrate. More income makes emigration easier and faster. So the platform has to deliver immediate earning value, and migration must be framed as a choice the professional is making, never as an exit we are helping them take or a betrayal we are talking them out of.

The six product pillars.

1. **Career Readiness Score.** Free, no account, fifteen questions. Produces a domestic employability percentage plus readiness percentages for UK, US, Canada and Gulf. Shareable card. This is the viral front door.
2. **Salary Intelligence.** Give to get. Complete a two minute salary survey to unlock the salary map. Cadre specific, facility specific, city specific. An annual CadreHealth Report is the press and backlink play.
3. **Hospital Reviews.** Anonymous verified reviews across eight dimensions: compensation, pay timeliness, workload, equipment, management, safety, training, accommodation. Give to get on the review text, with ratings always visible.
4. **Credential Wallet.** Licence and regulatory status tracking with reminders, a qualification vault, a CPD tracker with gap analysis, and a readiness dashboard by destination country.
5. **Locum Marketplace.** Shift posting and matching, instant pay, bundled indemnity. Phase two.
6. **Verified Anonymous Community.** Licence number verified against the regulator's register, then cryptographically separated from activity, so people can speak freely about their own facility. Private hospital, cadre and city channels.

Alongside these: a career advisor, a CV builder, a job board, mentorship, migration pathways and exam guides. The CV is the application document, not an upload.

Palette. Inherits Cadre Navy `#0B3C5D` and Cadre Gold `#D4AF37` from CFA, plus a feature colour system: Readiness Green `#10B981`, Salary Amber `#F59E0B`, Reviews Blue `#3B82F6`, Report Purple `#8B5CF6`, Advisor Pink `#EC4899`, Jobs Red `#EF4444`, CV Cyan `#06B6D4`, Wallet Indigo `#6366F1`, Migrate Orange `#F97316`, Exam Teal `#14B8A6`. A dedicated logomark is the highest priority open brand task in the firm.

Voice principles.

- **Talk to the cadre, not the system.** Every line should make a junior pharmacist or a third year doctor feel seen. No HR voice.
- **Honest, not aspirational.** We earn trust with real salary numbers and real reviews. Match the copy to the receipts.
- **Cadre inclusive by default.** Doctors are not the centre. Nurses, midwives, pharmacists, lab scientists, radiographers and community health workers are first class.

Say: real salary data, honest hospital reviews, verified credentials; for doctors, nurses, pharmacists and every cadre in between; your career, your call.

Avoid: talent, human capital, resources; AI career coach, AI matched jobs; migration framed as exit.

The state of the platform, which you must know before you plan any campaign. *Internal only.*

- 10,226 professional records held. 10,178 of them are medicine or dentistry.
- Only **660 have ever logged in**. 97 have verified an email address.

- The other roughly 9,500 are a cold import from an NMA list of 5,712 specialists brought in on 21 May 2026 plus earlier batches.
- The reviews platform launched in April 2026 with 191 hospitals listed across all 36 states and the FCT, and reached launch with zero reviews. It is a textbook cold start.
- A hard bounce rate of 11.09 per cent was recorded across roughly 19,000 sends, and the mail provider's bounce webhook had never been configured, so nothing was suppressed. Back filling suppressions is a precondition of any further bulk send.
- The imported name data is split wrong: the first name field holds the title and the last name field holds the middle name, so around 28 per cent of "Dr {lastname}" greetings are wrong. There is a helper for this and any send must use it.

So "every CadreHealth member" is two completely different populations. The dashboard reaches 660 warm users. Reaching the rest is an outreach decision with a deliverability problem attached, not a product decision.

Revenue today. A Pro subscription at N1,500 a month plus N5,000 per one to one session. One person has ever paid. He is a diaspora paediatrician who converted himself off a cold list, and he is the best single piece of evidence we have about what a paying member actually wants.

Revenue model in full. Placement fees on recruitment, locum margins, employer subscriptions, insurance distribution, earned wage access fees, workforce intelligence sold to ministries and multilaterals, CPD content partnerships, international recruitment referrals.

Recruitment pricing. The modal Nigerian market rate is twenty per cent of first year gross salary, with a three month replacement guarantee and eight to twelve week delivery. Our position is fifteen to twenty per cent with tier logic, three to six month replacement, six to ten week delivery. We undercut modestly on price and beat on platform leverage. *Internal only.*

Competition. ProLocums is the closest digital competitor and is early stage. The actual incumbent is WhatsApp groups. Nobody does salary intelligence, hospital reviews or credential management for Nigerian healthcare.

2.4 Maarova

What it is. A psychometric assessment and coaching platform for healthcare leaders, calibrated to the African context rather than translated into it.

Tagline. Assess, develop, and retain Africa's healthcare leaders.

Positioning statement. Maarova is CFA's proprietary psychometric assessment and coaching platform for healthcare leaders. Six dimensions, ICF certified African coaches, and a measurable arc from assessment to retention.

Audience. Hospital groups, training hospitals, health system foundations, boards and HR directors hiring or developing frontline, middle and executive leaders. The buyer is almost never the person being assessed.

The three streams. Every Maarova organisation is set up on one of three streams and the stream decides which modules run.

- **Recruitment.** Assessment as part of a hiring decision. "Hire with science, not gut feel."
- **Development.** Assessment as the opening of a coaching relationship. "Assess. Coach. Measure. Repeat."
- **Intelligence.** Assessment across a whole leadership population to see the landscape. "See your entire leadership landscape."

The six modules. Behavioural style, values, emotional intelligence, clinical leadership transition, culture, and a 360 feedback module. The clinical leadership module is deliberately restricted to clinical professionals, so a non clinical leader is not penalised for not having it. The 360 is decoupled from the core because it depends on external raters and takes days or weeks. Results appear progressively as each module completes, so a participant gets value immediately rather than at the end.

Palette. Maarova Night `#0F1A2A` and Warm Amber `#D4A574` as the primary pair. Stream Blue `#2D9CDB` for recruitment, Stream Green `#10B981` for intelligence. Report signal colours: Amber `#F59E0B` for caution, Signal Red `#EF4444` for critical findings and retention risk, Insight Purple `#7C3AED` for chart series. The wordmark ships as type only, **Maarova™**, alongside the CFA mark. A dedicated logomark is an open design task.

Voice principles.

- **Science, not horoscope.** Lean on validity, reliability and predictive language. Never describe results as personality types.
- **Coach led, not algorithmic.** Every output points to a human coach. The platform supports the relationship, it does not replace it.
- **Measured language for leaders.** Senior consultants and chief medical directors are the audience. Calm, respectful, developmental.

Say: psychometric science meets African healthcare; assess, coach, measure, repeat; paired with a certified coach from our African network.

Avoid: personality test, find your type; AI coach, AI generated insights; exclamation marks; hustle culture phrasing.

The methodology, which is our best and most underused content. This is the part that distinguishes Maarova from every off the shelf instrument in the market, and almost none of it has ever been published.

- The Hofstede scores that most consultancies still quote for Nigeria come from IBM employee surveys conducted between 1967 and 1973, and the Nigeria figure was derived from a three country West African aggregate rather than a Nigeria specific sample. Maarova treats Hofstede as background literature, not as a load bearing framework.
- The integrity backbone is HEXACO, whose Honesty Humility factor outperforms every Big Five dimension as a predictor of counterproductive work behaviour, and which has been validated in Nigerian samples.
- That is combined with the South African Personality Inventory, the most rigorous indigenously developed African personality assessment in existence, built across all eleven official South African languages from how native speakers describe personality rather than by translating a Western inventory. Its integrity factor weights fairness and resistance to discrimination, which fits the African pattern of integrity failure better than a greed weighted Western construct does.
- Response bias is handled through the GLOBE practice versus values gap, which measured each dimension twice, once as what people do and once as what people think should happen. The size of that gap predicts which items will pull answers toward the aspirational pole.
- The response format direction is quasi ipsative forced choice scored with Thurstonian item response theory, which is materially harder to fake than a Likert scale and supports valid comparison between candidates.

The Ubuntu coaching framework is the coaching differentiator. Goals are framed around collective impact rather than individual achievement. The coaching question is "how does your leadership serve your community". It respects power distance by positioning the coach as a thought partner rather than an authority. No validated Ubuntu leadership instrument exists anywhere, which is an opportunity, and it is ours

to take.

Coaching evidence worth quoting. The single strongest predictor of coaching outcomes is the working alliance, the quality of the relationship, at roughly r equals .41, which is more predictive than credentials, session count or methodology. Mayo Clinic found that six coaching sessions significantly reduced physician emotional exhaustion. Industry return on investment is reported at five to eight times when measured on productivity and retention.

Coaching product shape. One to one is twelve sessions over six months, biweekly, video plus WhatsApp. Group coaching is four to six peer leaders monthly over three to six months. On site workshops run half or full day for ten to thirty leaders. Leadership retreats run two to three days offsite for ten to twenty. An annual conference for fifty to two hundred is the thought leadership vehicle. Coaches are sourced from the certified coaching bodies across Nigeria, South Africa and Kenya, with a target acceptance rate of ten to fifteen per cent as a quality signal, and a fifteen minute chemistry call before any engagement starts, which is non negotiable.

Pricing anchors. *Internal only.* Lagos Business School's Advanced Management Programme is N17 to 18M per person. Its Chief Executive Programme is N26 to 27M. Harvard's global health leaders programme is 45 to 50 thousand dollars. Against that, an enterprise cohort of ten leaders costs a hospital about the same as sending one person to Lagos Business School. That comparison is the sales argument and it is a strong one.

Where Maarova is actually deployed today. Five founders of a paediatric facility under a dedicated organisation on the development stream, as part of a board level alignment read. A faith based organisation's staff body, with a coach trialling the instrument first and an administrator tracking completion but never seeing individual results. Candidate assessment inside recruitment mandates for a healthcare group. That is the live footprint. It is small and it is real.

What is not yet true, and must not be claimed. *Internal only.* A diagnostic run in May 2026 on nineteen completed sessions found four things. The emotional intelligence scale is not discriminating, with a median of 95 and a floor of 80, which means it is measuring social desirability as much as emotional skill. The 360 funnel collapses, with 72 per cent of subjects who created a 360 request never sending a single rater invite. There is no integrity construct in the current six modules at all, so integrity vetting is not a claim the current instrument supports. And response time is not captured on any item, which removes the cheapest available check on careless responding. These are being fixed as version one updates. Until they are, do not write anything that claims predictive validity, normative percentiles or integrity measurement.

2.5 Mezo Health

What it is. A verified specialist network and a hospital operating stack. A doctor consults at network facilities or by teleconsultation, and Mezo fills the clinic, runs the schedule and collects the payment. It is the answer to a specific and very common problem: a good doctor who wants a private practice does not want to become a landlord, a receptionist, a billing clerk and a claims chaser in order to have one.

Status. Mezo Health has no separate company registration, so it runs as a Consult for Africa product and service offering. In client facing material it is named as **CFA's facility operations practice**. It has its own codebase, its own database, its own authentication, and lives at mezohealth.com.

What it does for a doctor. Facility connections so a doctor can consult at rooms they do not own. Schedule and clinic management. Payment collection. Claims processing with the health plans. On the growth plan, up to eighty per cent advance factoring against pending claims, which is the line that matters most, because the reason Nigerian private practice strangles good doctors is that the money is real but it arrives late.

Plans. Starter, Growth and Diaspora. The diaspora plan exists because a Nigerian specialist abroad can hold a practice presence here without relocating.

What it does for a facility. Room and session utilisation, through a model of facilities, rooms, access rights, group room contracts and visit blocks. An underused consulting room is pure margin once it is filled.

Patient side. A membership product.

Eligibility. Mezo runs on the MDCN register, so places are open to medicine and dentistry only. Of the CadreHealth records held, 10,178 of 10,226 qualify, so the constraint costs almost nothing. The offer is hidden from ineligible cadres rather than shown and then refused.

The positioning rule, and this is the important part. Sell Mezo as a **private practice opportunity**, and sell it aspirationally. The line is "your own practice, without building one". It is never "claim your account", never "sign up", never "register". The audience is senior clinicians who have been pitched by everybody, and consumer application language reads to them as junior. See the senior medic copy rules in Part Three, which apply to Mezo more than to anything else we write.

How it connects to CadreHealth. A member answers a private practice survey on CadreHealth. Their answers are stored first, because they are the thing that cannot be recreated. CadreHealth then asks Mezo to open a place and hands the member a link. The member sets their own password and their real MDCN folio number at that link and Mezo takes over onboarding. Nobody is verified by any of this: registration verification and indemnity remain the doctor's responsibility and Mezo's job. If Mezo is unreachable, the survey still counts as answered and the member is told their place is being opened rather than being asked to fill the form in again.

Brand assets. Mezo has no brand guide yet. It currently runs on its own product styling. If you are asked to produce Mezo marketing material, the brand definition is an open piece of work and should be scoped before any campaign.

2.6 Brand hygiene, the open items

These are the known gaps. Several of them are things you may be able to close in your first quarter.

- CFA has no SVG logo master. PNG only.
- CadreHealth has no logomark. It renders as type on CFA navy and gold. This is the highest priority open brand task.
- Maarova has no logomark. Type only, Maarova™.
- Mezo Health has no brand guide at all.
- Typography is unresolved on all three brand guides. All currently use the system stack.
- oncadre.com is the intended CadreHealth domain and is not yet registered. Defensive domains flagged: cadrehealth.africa, cadrehealth.ng, cadre.africa.
- Naming inconsistency to fix firm wide: Med Lyfe, Medlyfe, The Lyfe Place and The Lyfe Centre are used interchangeably in client material.
- A credential claim appears as both "60 plus years" and "135 plus years" of combined experience in different documents. Pick one and make it true everywhere.

Part Three: Positioning, targeting and the rules

3.1 The market we are speaking into

Do not write a single line of content without these numbers in your head. They are the context every reader in this market already lives inside, and a piece that ignores them reads as written by an outsider.

The workforce. Roughly 80,000 doctors are registered with the Medical and Dental Council of Nigeria and more than 200,000 nurses with the Nursing and Midwifery Council. Around 16,000 doctors emigrated in five years against roughly 55,000 licensed, and 8,560 registered with the UK General Medical Council between 2021 and 2024. Around eighty per cent of Nigerian healthcare workers say they intend to emigrate. There are perhaps 10,000 to 15,000 Nigerian trained doctors abroad.

The training bottleneck. About 6,000 medical graduates a year against accredited housemanship capacity of roughly 4,000, which strands around 2,000 graduates annually. There are 114 accredited hospitals, made up of 47 teaching hospitals, 21 federal medical centres, 37 general and specialist hospitals that are mostly state and military, and just 9 private. Only 44, all federal, are on the central portal, so 70 sit outside it. Teaching hospitals route only around forty per cent of their places to the open portal and reserve the rest. A bribe of about N400,000 for a slot has been reported publicly. These figures are attributable to the MDCN Registrar through Senate testimony and national press, which makes them safe to cite.

Medical tourism. Nigerian outbound medical travel is tracked at around 685 million dollars and estimated at close to 2 billion. Orthopaedics is one of the top leakage specialties. The barrier to keeping that spend at home is trust, not price. The cheapest joint replacement in Nigeria now sits at a floor of about N5.5M after devaluation, while an Indian package at four to eight thousand dollars is N6M to N13M for the surgery alone before travel, so a true trip abroad costs more than ten million naira. Local is already better value and more convenient. It is not believed.

Outreach benchmarks. A generic cold email to a physician gets a 1 to 5 per cent reply rate. A personalised, senior targeted one gets 20 to 40 per cent. Personalised subject lines lift open rates by around half. Subject lines of 36 to 50 characters perform best. In the 2025 BMJ consultant survey only four per cent were actively job seeking while 47 per cent were open to changing jobs. Write for the passive forty seven, never the active four.

3.2 Who buys what

This is the buyer map. Everything you write should be aimed at a row in this table.

Buyer	What keeps them awake	What we sell them	Where they are
Hospital owner or promoter	Money in, money not coming out; a building that is not yet a business	Diagnostic audit, feasibility, turnaround, build and operate	LinkedIn, referral, industry bodies
Group chief executive	Business unit leads they cannot direct; expansion they cannot resource	Group strategy office, embedded team, venture structuring	LinkedIn, direct, peer referral
Board chair or non executive	Not knowing whether what they are told is true	Organisational audit, board governance, director training	Peer referral almost exclusively
Medical director	Clinical risk they can see and cannot fix alone	Clinical governance, safety culture, standards of work	LinkedIn, medical associations
HR or people director	Hiring senior people who then fail	Recruitment, Maarova assessment, coaching	LinkedIn, HR communities
Specialist in private practice	Income that depends entirely on their own hours	Mezo, positioning and referral engine work	Direct, WhatsApp, peer referral
Health plan or HMO executive	Provider network that is long but not adequate	Network strategy, provider acquisition	Direct, industry bodies

Buyer	What keeps them awake	What we sell them	Where they are
Individual professional	Being underpaid and not knowing it	CadreHealth	X, Instagram, WhatsApp, search
Diaspora specialist	Wanting to contribute without moving home	CadreHealth diaspora, Mezo diaspora plan, DFC	LinkedIn, diaspora associations
Development financier or ministry	Programmes that do not deliver	Health systems strengthening, workforce intelligence	LinkedIn, conferences, publications

3.3 Positioning against the field

Against the international strategy firms. They arrive with a framework and leave with a report. We have run hospitals in this market and we stay to operate. Our proof is that several engagements have progressed from advice to delivery to management. Never name a competitor.

Against local boutiques. Most are one or two people selling the founder's time. We have products, data and a bench, and our diagnostics are instrumented rather than interview led.

Against software vendors. They sell a system and call the outcome your problem. Our platforms exist because our consultants needed them, not the other way round, which is why they are never the lead in a client conversation.

Against recruitment agencies. They fill the role. We assess whether the person can lead, place them, and then have to live with the result inside a client we are also advising.

The single sentence that does the most work: we do not hand you a report and walk away.

3.4 Content pillars

Consult for Africa. LinkedIn is primary and almost everything else is secondary.

Pillar	What it looks like
Method and thought leadership (40%)	The case studies in Part Four, market analysis, regulatory commentary, policy
Case studies and results (25%)	Anonymised outcomes, before and after, engagement spotlights
Team and culture (15%)	Consultant profiles, the diaspora bench, how an engagement actually runs
Industry commentary (10%)	Reaction to policy, workforce data, investment news
Hiring and network (10%)	Open roles, consultant recruitment, partner firms

Cadence: LinkedIn four times a week, X daily, Instagram three times a week.

CadreHealth. X and Instagram are primary. Medical Twitter in Nigeria is large and active.

Pillar	What it looks like
Salary intelligence (30%)	What a registrar in Lagos actually earns, public versus private, salary scale breakdowns
Career guidance (25%)	Readiness, interview and CV advice, exam and migration pathway guides

Pillar	What it looks like
Facility transparency (20%)	Review summaries, what your hospital will not tell you, facility comparisons
Community and identity (15%)	Professional pride, cadre specific content, mentorship stories
Product (10%)	New features, readiness score prompts, member stories

Cadence: X two to three times daily, Instagram five times a week, TikTok three times a week, LinkedIn twice a week.

What reliably performs: salary transparency, "which hospital pays better" comparisons, migration pathway threads, "things I wish I knew as a house officer", exam preparation, review summaries.

Maarova. LinkedIn almost exclusively, plus email nurture.

Pillar	What it looks like
Workforce and retention data (35%)	Attrition costs, turnover economics, the real price of a mis hire
Assessment science (25%)	The methodology in section 2.4. This is the untapped seam.
Client results (20%)	Anonymised outcomes, leadership alignment findings
Product education (10%)	How the streams work, what a report contains
Thought leadership (10%)	African leadership development, Ubuntu framework, cultural context in assessment

Cadence: LinkedIn three times a week, email monthly plus triggered nurture.

Mezo Health. Direct and referral first. Social is secondary and should be run only once the brand definition exists.

3.5 The rules. Read this section twice.

These are not style preferences. Several of them exist because of a specific incident.

3.5.1 Never use an em dash.

Not in headlines, not in body copy, not in bios, not in user interface text, not anywhere. Use a comma, a full stop, a colon, or restructure the sentence. This is a firm wide rule and it is checked.

3.5.2 Never lead with AI.

Do not write "AI powered", "AI driven", "AI suggestions", "AI coach" or "AI matched" in anything a user or a client will read. Our audience hears data privacy risk, not capability. Describe the outcome, not the engine. "Intelligent analysis", "personalised insights" and "career advisor" are the approved alternatives.

3.5.3 Never name our platforms in client body copy.

In a proposal, a report or any client facing document, describe what will be done rather than which product does it. Write "a structured leadership assessment on every shortlisted candidate", not "the Maarova leadership assessment". Write "market mapping and direct approach, including senior Nigerians working abroad where the role suits a returner", not "sourcing through CadreHealth plus the diaspora bench".

The single exception is an appendix the client has specifically asked for, where the platforms are named in one place the client can choose to read. Named once in an appendix is fine. Sprinkled through body copy as credentials is not.

This rule does not apply to marketing material for the platforms themselves. Obviously CadreHealth copy names CadreHealth. The rule governs what goes into a consulting deliverable.

3.5.4 Never name a client without written clearance.

Every case study in Part Four is anonymised and the anonymisation is deliberate. The only client name currently cleared for public use is the rehabilitation centre proof point in section 2.2, because it has already run in our own advertising. Everything else, including logos, including "a leading Lagos hospital" phrasing that is specific enough to identify, needs Debo's explicit sign off.

3.5.5 Writing to senior medics.

This audience receives constant low quality pitches, and copy that respects seniority is the entire differentiator.

Lead with their experience, not the platform. Say "you have", not "we offer".

Phrases that work:

- "Colleagues like you"
- "Your name appears on the [specific register]", which is factual rather than flattering
- "We are reaching a small group of senior consultants"
- "Whether you are practising in Lagos, in London, or have stepped back from full time clinical work"
- A peer to peer signature: "From Dr Debo Odulana, President, Doctors Foundation for Care"
- A specific time bound: "Twenty minutes will let you..."

Phrases to avoid:

- "Job opportunities", which assumes unemployment
- "Career advancement", which is junior coded
- "Exciting opportunity", which reads as recruiter spam
- "Sign up" and "register", which are consumer application coded. Use "claim your profile" or "confirm your record".
- "Earn extra income", which is money first and insulting to a senior clinician
- "We are excited to invite you". Write "We are writing to you".

For retired physicians, status is the motivator and money is secondary. Named designations, public recognition and peer society are what senior fellows' clubs in the UK royal colleges actually sell, and members pay to retain their designation after retirement. Lead with something like an Emeritus Fellow designation before any mention of money, and frame the offer as "share your knowledge", never "earn extra".

For the diaspora, default to non prescribing engagement: mentorship, teaching, case review, advisory. Anything requiring Nigerian relicensing is opt in and never the default. The evidence on this is unambiguous. Permanent return programmes consistently underperform. Israel spent 120 million shekels, registered 4,424 people, brought 1,000 home and found jobs for 226, then closed the programme. Ghana's health placement programme achieved 65 placements over a decade by doing the opposite: temporary placements with named host hospitals in defined geographies. Never lead with "come home".

3.5.6 The client report voice, for anything written in Debo's name.

Short paragraphs, one idea each. Declarative sentences that state a position and stop. No contractions. First person singular for judgement calls, first person plural for the firm, second person for the client.

He writes **long, connected sentences joined with because, so and which**, explaining as he goes. He does not write clipped aphorisms. "You can be twenty six per cent over budget. You can be eight months

late. You cannot be both." is copywriting and it was rejected. Staccato declaratives and "not X, but Y" constructions read as machine written even when every word is correct.

Never put a question, a brief or a concern in a client's mouth. If a framing came from Debo, write it as his own thinking: "I have been thinking about your site since", not "you asked whether".

He signs **Warm regards, Dr Debo Odulana**. He is a doctor. Not "Debo Odulana".

Before you draft anything in his name, read something he actually wrote. The reference document is `docs/haven-cover-email-kabir.md` in the repository. Do not infer the voice from the subject matter.

3.5.7 Campaign mechanics that will bite you.

Internal only, and each of these is the residue of something that went wrong.

- **All bulk and outreach email goes through the ZeptoMail HTTP API.** Never through Zoho SMTP. Zoho throttles batches and returns "unusual activity" failures partway through a send, which leaves a campaign half delivered with no record of where it stopped.
- **hello@ is the sender of record.** A send that hardcodes a platform address leaves no evidence in the mailbox Debo checks.
- **Verify the destination before any bulk send.** A live page can sit in front of a dead endpoint, and a 200 response can sit in front of a parser that read nothing. The discipline is to post the real payload, read the row back, assert the fields survived, then delete the test rows. Assert on the stored record, not the status code.
- **Suppressions must be back filled before the next CadreHealth send.** See section 2.3.
- **Use the salutation helper for any CadreHealth greeting.** The imported name fields are transposed and roughly 28 per cent of naive greetings are wrong.

3.5.8 Sensitive material.

Some of our engagements touch clinical incidents, regulatory exposure and family disputes. There is a standing rule across the firm that a consultant's written document is discoverable and can travel anywhere. Client facing documents have been deliberately expunged of anything that could implicate a client or the firm in a regulatory or legal proceeding.

The marketing consequence is absolute. **No clinical incident, no patient outcome, no mortality, no regulatory finding and no named individual ever appears in any external content.** Where a case study touches this territory, Part Four says so explicitly and tells you what the permitted framing is. If you are unsure, the answer is no, and then ask.

3.6 The proof ladder

We have more proof than we are using, and it is worth understanding it as a ladder, because different assets serve different stages.

1. **Method proof.** We can show how we work without showing who for. This is the largest and most underused seam and it is what Part Four exists to unlock.
2. **Data proof.** Workforce numbers, salary data, review data. Ours, defensible, and nobody else in the market has it.
3. **Outcome proof.** Anonymised results. Available on several engagements, needs clearance per figure.
4. **Named proof.** Currently one cleared reference. Getting two or three more named references is the highest value business development task available to marketing, and it is worth asking for them at the point an engagement closes well rather than months later.
5. **Person proof.** Debo's own record. Ran a hospital group turning over more than a billion naira a month. Took over a million dollars of cost out of a major private hospital and lifted revenue 23 per cent in

three months through pay for performance. Built and sold a health technology business. This is publishable and is currently underused in firm content.

3.7 Search and programmatic

Two search plays are already designed and partly built.

CadreHealth hospital pages. A page per facility with salary data and review summaries, with structured data and dynamic social images. The search intent behind "what does X hospital pay" is enormous and almost entirely unserved.

Pay scale content. The public sector salary scales are among the most searched terms in Nigerian healthcare and are pure acquisition content.

For CFA itself, the insights section of the site is the home for long form. The case studies in Part Four should each become an insights article before they become anything else, because an article can be linked from LinkedIn, sent to a prospect and indexed, and a post cannot.

Part Four: The case study library

4.0 How to use this part, and the anonymisation standard

Ten engagements, written in case study form, with the methodology shown rather than summarised. Each one is the raw material for an insights article, a LinkedIn series and, in several cases, a conference paper.

The anonymisation standard. A client is described by **sector, setting, scale and stage**, never by name, owner, location more precise than a city, or any detail that would identify them to somebody in the market. "A five bed paediatric unit with a three cot neonatal intensive care service in Lagos, fifteen months old at the time of the engagement" is the correct level. "A paediatric hospital in Ikeja founded by a group of five" is not, because the market is small enough that it names them.

Figures. Ratios, percentages and multiples are generally safe. Absolute naira figures attached to an identifiable client are not. Where a figure below is marked publishable, it is because the ratio carries the point without the client's revenue attached to it.

The four line test before anything goes out. Could a competitor identify the client from this. Could the client's staff identify themselves from this. Does anything here touch a clinical outcome, a regulator or a named individual. Would the client be pleased to read it. If any of the first three is yes, or the last is no, it does not go out.

4.1 The facility whose revenue record was a nursing handover sheet

Organisational audit and growth strategy. Paediatric and neonatal inpatient care, Lagos. Five general paediatric beds and three neonatal intensive care cots. Fifteen months old at engagement. Founder owned, five founders, board led.

The brief we were given

The owners wanted help with operational optimisation, growth and margins, and with stock and procurement. They described a facility that was busy, well regarded by its patients, and yet seemed to have nothing left at the end of each month after salaries were paid. The presenting question was essentially "where is the money going".

What we found

Three findings reordered the entire engagement.

One. It was a working capital problem, not a profit problem. Roughly one period's worth of revenue was sitting outside the business, split between receivables owed by health plans and stock sitting in the pharmacy. "There is barely anything left after salaries" is what a working capital trap feels like from inside.

The business was not unprofitable. It was illiquid, and those require completely different interventions.

Two. The revenue record was a nursing handover document. The standard operating procedure instructed nurses to bill opened medications from memory at the end of a shift and to fill in the revenue spreadsheets before handover. That single instruction explained every downstream data finding we had: an entire month of health plan billing missing, private revenue understated in one month by more than a million naira, and pharmacy stock failing to roll forward in every month tested. We had spent weeks treating these as separate data quality issues. They were one procedural issue with many symptoms.

Three. The establishment did not match the service. Counting from the actual rotas rather than the organisation chart, the nursing establishment for the first half of the year was smaller than the reports claimed. No pharmacist was rostered on any day in one month tested, which meant paediatric weight based dose verification was being done by technicians. Several days were covered by two people across twenty four hours, and on some of those it was one registered nurse plus a technician who was simultaneously double booked elsewhere. A generator was recorded as down for six consecutive months while critical services ran on a smaller unit.

And the finding that changed the maturity rating: **the verdict flipped direction between evidence streams.** Interview evidence alone would have caused us to upgrade the revenue cycle, because the people we interviewed described automated billing, a very high clean approval rate and fast refunds. The documents forced it back down. Overall organisational maturity landed at 2.5 rather than the 2.9 the interview only draft had carried. Patient facing workstreams rose. Everything supporting them fell.

The method

Four independent evidence streams, deliberately chosen so that no two could fail the same way.

1. **Field validation interviews** across nine departments, conducted on site by an analyst, structured by role.
2. **A staff safety culture survey**, adapted from a validated international hospital survey instrument with locally written items added, fully anonymous, delivered as a mobile first web form and a printed instrument with a QR code. Twenty three responses over a fortnight.
3. **A patient and caregiver experience survey**, same delivery, sixteen responses. This was the first time the patient voice had ever been measured at this facility, and it scored the highest of any stream.
4. **Documentary analysis of thirty four documents** from the client's own drive, worked into consolidated evidence packs producing seventy four discrete raw findings, each with a reference.

Ten domains, structured so that nothing overlapped and nothing was missed. Governance, finance, revenue cycle, supply chain, people, clinical standards, operations, data and reporting, patient experience, growth.

A formal maturity assessment, scored per domain, so that the client had a baseline to be measured against later rather than a list of complaints.

A correction discipline that is worth copying. Two of our three headline findings had to be partially corrected before delivery, because the evidence packs had inferred absence from documents that simply had not been supplied. A registration was recorded as "not in the documents" and written up as "does not exist", and that was wrong. A consultant rota existed and had not been sent. We corrected both before the report went out, and we changed the standing instruction to the team: **check any finding of the form "X does not exist" before repeating it.** An audit that cries wolf once loses the room for everything else in the document.

What it changed

The engagement progressed from a diagnostic into a growth mandate with a specific target and a set of levers. The tariff finding alone justified it: a very large majority of the facility's several thousand line price list was priced at zero, and the lines that were priced averaged around a third below comparable local providers. That is not a pricing tweak. That is an entire pricing architecture that had never been built.

The board was given **decisions with options** rather than recommendations: growth axis, operating model, workforce proposition, service portfolio and funding, each scored on speed to cash, capital required, management strain and whether it relieved the actual binding constraint.

The transferable principle

Reporting integrity is a systems finding, not a character finding, and it must be written that way. Two functions inside this business reported opposite facts on the same question to the same board in the same month, and nobody reconciled them. Our report stated the facts, noted that each had an individual explanation, said that the overall shape required explanation, and recommended an independent reconciliation performed by somebody outside the operations function. It referred to people by role throughout and named nobody. That is the difference between a board that acts on a report and a board that defends against it.

Publication guidance

Article angle. "Four evidence streams, and why the documents outvoted the interviews." A genuinely original piece about diagnostic method, with the maturity score movement as the hook.

LinkedIn hooks.

- "We were about to upgrade a client's revenue cycle. Then we read their nursing standard operating procedure."
- "If your nurses are billing from memory at the end of a shift, your revenue record is a handover sheet. Everything downstream follows from that one line."
- "The interviews said one thing. The documents said another. Here is why we went with the documents."

Cleared to publish. The method, the four stream design, the ten domain structure, the maturity movement as a direction rather than a score, the tariff findings as proportions, the working capital insight as a principle, the correction discipline.

Never publish. *Internal only and absolute.* Anything relating to clinical incidents or patient outcomes at this facility. Anything about its regulatory registration status. Any absolute revenue, receivable or stock figure. Any reference to the number of founders, the facility's name, its district, or the fact that it is fifteen months old combined with any other identifier. The reporting integrity material may be discussed only as the general principle in the section above, never as a finding about a specific client.

4.2 The surgeon who blamed his postcode

Positioning and organisational audit. Specialist orthopaedic practice, mainland Lagos. Single principal surgeon, UK trained, subspecialised in joint replacement, revision and complex trauma. Owner operated clinic plus visiting privileges at two private hospitals.

The brief we were given

The surgeon believed his growth was constrained by his clinic's location and environs. He wanted help moving, or help making the location work.

What we found

Location was a symptom and not the disease.

Elective joint replacement is a trust and referral purchase, not a footfall purchase. Nobody walks past a clinic and decides to have a hip replaced. The binding constraints were a thin referral pipeline, a brand that was

under sold relative to the surgeon's actual credentials, and medical tourism leakage that was never being recaptured. A different postcode would not have touched any of the three.

The second finding, which came out of the audit that followed, is the one worth writing about. **The practice was bigger than the company.** A meaningful share of the principal's income was earned in somebody else's theatre and invoiced outside his own company, while his company carried a fixed cost base for a surgeon who was frequently elsewhere. Every strategic promise we had helped him make, about published outcomes and fixed price bundles and complex case authority, was a promise about the business, and none of it could be executed until the organisation underneath was known.

We said this to him in the engagement letter, up front, rather than saving it for the report.

The method

The ten case trace. Ten real cases, selected on the morning of the site visit with the principal present so that nothing could be curated in advance: three primary joint replacements, two revisions, one infected joint, two trauma cases, one complication and, critically, **one case that was quoted and never proceeded**.

Each case was then followed through a twenty two link chain from first enquiry to current outcome. Clinic note. Decision to treat. Quote. Deposit. Pre operative assessment. Consent. Theatre register. Safety checklist. Antibiotic timing. Implant lot stickers. Sterilisation load record. Operation note. Observations. Discharge. Invoice. Collection traced to the bank statement. Claim. Follow up attendance. Outcome.

Twelve numbers were captured per case, which produced the first honest case economics and funnel timing the practice had ever had.

Tracing ten cases completely beats sampling a hundred partially. Every gap found is specific rather than theoretical. You are not saying "documentation is inconsistent". You are saying "in this case, on this date, the implant lot sticker is not in the notes, so if that batch were recalled tomorrow you could not find this patient".

Four survey instruments, all anonymous, all delivered as both web forms and printed instruments with QR codes so the paper and the web could not drift apart: staff culture, patient experience, referrer, and leadership direction.

The referrer list as an output rather than an input. This is the finding we are proudest of methodologically. We asked for a list of referring doctors. He did not have one. That absence is itself the finding, so asking him for the list was asking him to solve the problem the audit existed to find.

So we built the list three ways instead. The referrer survey travels by forwarding and asks every respondent to name up to three more colleagues, with separate consent on whether we may say who sent us. The patient survey asks patients who sent them, because patients remember the referring doctor even when the clinic never recorded it. And the fieldwork treats the referrer harvest as a deliverable, mining "referred by" fields off registration forms and first clinic notes across twenty four months, then deduplicating and segmenting by date of last referral.

The referrer survey also asks each colleague whether their referring has risen, fallen or stopped, with an open box for why. **The doctor who quietly stopped referring is the most valuable conversation available to this practice, and no handed over list would ever have found him, because a list of current referrers is precisely the set he has already left.**

One link, not six attachments. Everything the client and his team needed sat behind a single URL: the two client documents, the priority information requests, a secure uploader, the four surveys, the print pack and what would happen on the day. The internal fieldwork kit was deliberately excluded from it, because that document names what we intend to probe and the link goes to the whole team.

What it changed

The positioning work that preceded the audit produced a strategy built on making the competition irrelevant rather than beating it. Lead with complex case, revision and salvage authority, positioned as the surgeon other surgeons refer to. Voice it ethically, on when not to operate. Then four ranked value innovation moves, of which the first was radical transparency: publish real outcomes, real volumes and a fixed price. The evidence for that move is striking. Around ninety six per cent of patients say they want published surgical outcome data, fewer than half can find any, and nobody in this market publishes it.

The audit then existed to establish whether those promises could actually be kept and proved.

The transferable principle

The client's diagnosis is data about the client, not data about the problem. When a principal tells you the constraint is the building, that tells you what they have already considered and ruled in. It rarely tells you what is binding. The consultant's job is to accept the brief, take it seriously, and then test it against evidence the client has not previously looked at.

Publication guidance

Article angle. "The ten case trace: what following ten patients end to end tells you that sampling a hundred cannot." This is a genuinely original methodology piece and there is nothing like it published in this market.

LinkedIn hooks.

- "He asked us to help him move his clinic. We told him the problem was not the postcode."
- "Elective surgery is a trust purchase, not a footfall purchase. No one has ever walked past a clinic and decided to have a hip replaced."
- "We asked for his referrer list. He did not have one. That was the finding."
- "The doctor who quietly stopped referring to you is the most valuable conversation you are not having. A list of current referrers is exactly the set he has left."

Cleared to publish. The ten case trace in full, including the twenty two links and the case mix logic. The referrer as output insight. The location reframe. The published outcomes market statistics, which are from public research and not from the client. The trust versus footfall argument.

Never publish. Any clinical finding about this practice, including anything about sterilisation, implant traceability or infection. The principal's identity, his hospital affiliations or his subspecialty combination, which together identify him. Any revenue or case volume figure. The boundary finding about income earned outside the company, which is commercially and personally sensitive and is internal only.

4.3 The building that was two hundred square metres bigger than the brief

Feasibility, business case and build and operate mandate. Conversion of a four structure residential compound into a private ambulatory medical campus, Abuja. Client is a multi business healthcare group.

The brief we were given

Convert a residential compound into an ambulatory medical campus, theatre on the ground floor, family practice above. Let the consulting rooms by the session to recruited consultants. The group's chief executive had already paid two years of head rent in advance and wanted her investment back within a year.

What we found

The brief was wrong about the building. The as built drawings we were given were vector CAD exports, which meant the geometry could be recovered from them rather than trusted from the summary. We wrote a script that recovered walls, door swings and window symbols to an accuracy of about ten millimetres, and measured every space directly.

The footprint was not the dimension the brief carried. The theatre room was almost a third larger than stated, because the as built depth was two metres greater than the brief recorded. Reception was substantially larger. Gross area came out at nearly nine hundred and fifty four square metres against a brief figure of seven hundred and eleven.

And the important consequence: **the extra area was almost entirely circulation.** Corridor. Which meant the rent roll did not improve at all, and the fit out cost rose by tens of millions of naira. A measurement error had been silently sitting inside the business case in the direction that costs money and returns nothing.

The business model was also wrong, and for a reason that has nothing to do with property. A landlord model needed roughly sixty individual consultants recruited over twenty four months to fill the building. The alternative, moving the group's own clinical businesses into it, needed about twenty five, because six businesses already inside the group could simply move in. **The ramp, not the rate card, is what decides whether money comes back in a year.**

The second argument was that the group was going to have to manage those businesses anyway. Co located, they share one front office, one nursing pool, one billing function and one diagnostics service. Separated, each needs its own, which is roughly fifty to seventy million naira a year of duplicated overhead.

Modelled properly, the campus configuration returned more than double the landlord model to the group.

Two findings inside the model were counterintuitive and both survived challenge. The theatre pays fastest, not slowest: hire income less running cost, plus the day case uplift it gives the aesthetics business, is roughly a one year payback against its capital. The original business case had held the theatre back as the least certain element. We reached the same conclusion, gate it on a signed surgical anchor, for the opposite reason.

And the honest one. **A twelve month payback from the day the doors open is not available on this building.** Measured from stabilisation it is about a year. Measured cumulatively from opening it is twenty to twenty four months. The floor is around thirteen months and only with all three levers pulled: a reduced fit out, opening the first floor while the ground floor is still building, and pre selling the annual product lines. We said that plainly rather than fudging it, and the client respected it.

The method

- **Recover the geometry, do not accept it.** Where drawings exist as vector files, the real dimensions are in the file.
- **Model monthly, not annually.** The annual model said payback in twenty one months and peak funding of a certain size. The monthly model said twenty nine months and a peak funding requirement roughly twenty per cent higher, with the cash trough in month fifteen. Timing and funding questions cannot be answered by an annual model, and the difference is the difference between a venture that survives and one that runs out of money in the middle.
- **Size rooms, not just people.** The family practice model in the brief sized physicians and never sized rooms. Loaded properly it needed between two and five consulting rooms against the one the brief allowed, which collapsed the sessional pool from six or seven to about three and a half and cut the restated contribution by roughly a fifth.
- **Publish a complete establishment schedule with a "who pays" column.** Costing that properly found campus operating costs understated by tens of millions a year.
- **Name the open questions that change the answer rather than the confidence.** Two were flagged and left open in writing. Whether the facility can join the visa medical panels, which gates a substantial revenue line and is answered by one phone call rather than by marketing. And a contradiction between two of our own documents, where one carried a sessional fill assumption of thirty five to fifty five per cent and another carried a seventy five per cent booking ceiling, a 1.8 times difference on the largest revenue

line in the model. We said it had to be reconciled in one place before any campus number was quoted anywhere.

The transferable principle

Measure the asset before you model it. And when your own documents disagree with each other, say so in writing before the client finds it.

Publication guidance

Article angle. "Your feasibility study is only as good as your floor plan." The geometry recovery is unusual, concrete and highly credible, and it demonstrates technical capability without naming anybody.

LinkedIn hooks.

- "We recovered a building's real dimensions from the CAD file. It was two hundred and forty square metres bigger than the brief said, and every extra metre was corridor."
- "An annual model told our client payback in twenty one months. The monthly model said twenty nine, with the cash trough in month fifteen. Annual models cannot answer timing questions."
- "The theatre paid fastest, not slowest. We reached the same recommendation as the original business case for exactly the opposite reason."
- "We told a client that the twelve month payback she wanted was not available on that building. She thanked us for it."

Cleared to publish. The geometry recovery method and the fact that the extra area was circulation. The ramp versus rate card argument. The monthly versus annual modelling point. The room sizing insight. The duplicated overhead logic, as a range. The honesty about payback.

Never publish. The client, the group, the city combined with the building description, any absolute revenue or capital figure, and the identity of any of the six businesses.

4.4 The chief executive who would not override her own managers

Group strategy and multiple business unit mandates. Diversified healthcare group, Nigeria. Six business units plus a hospitals division.

The brief we were given

The group chief executive was frustrated with her business unit leads and wanted the group to perform better. The obvious read was a top down group transformation mandate addressed to her.

What we found

She would not override her business unit leads' autonomy, despite complaining about them. That was not a negotiating position or a weakness to be worked around. It was a fixed constraint, and it reshaped the entire commercial approach.

So we did not write one proposal. We wrote several, split by **who actually decides**.

At group level, which is her decision alone: an embedded strategy office sitting with the office of the chief executive, framed explicitly as support that does not override unit autonomy. Senior recruitment as a structural resourcing decision. Corporate shareholdings in new ventures.

At unit level, each proposal addressed to that unit's own leader, going via the office of the group chief executive rather than from it, with an explicit autonomy note in each one: support, not a takeover, and the unit head owns the decision.

The earlier attempt, a single eighteen page group proposal pitched top down to the chief executive, was superseded. It was a good document aimed at the wrong decision structure.

The method

Map the decision rights before you map the opportunity. For every piece of work identified, ask three questions. Who signs. Whose consent is required. Whose cooperation is required for delivery even though they do not sign. Then write one document per signer.

Respect the constraint in the artefact itself, not just in the conversation. The autonomy note in each unit proposal was not diplomacy. It was the thing that made the proposal acceptable to a unit head who would otherwise have read it as the chief executive sending consultants to audit him.

What it changed

The group level centrepiece was approved. So were multiple ventures, including a new clinical campus, a set of special purpose vehicles and two recruitment mandates. A joint venture with a visiting specialist was restructured from a full company formation into a short pilot, on the reasoning that the relationship should be tested before it is papered.

A commercial finding worth carrying forward came out of this engagement. A flat percentage of revenue management fee sits above the norm and is the structure the market likes least. Restructured into a base percentage that funds the operating team plus an incentive share of profit that is subordinated to the client's priority return, the economics are almost identical if it performs, and it is far more defensible. **Propose the split yourself rather than waiting to be pushed into it.**

The transferable principle

Sell into the organisation's actual decision structure, not its organisation chart. The chart says the chief executive decides. The reality was that six people did, and a single proposal addressed to one of them would have been rejected by five.

Publication guidance

Article angle. "Who actually decides? Why one proposal to the chief executive is usually the wrong document." This is unusually good business development thought leadership and it will resonate with every group chief executive who reads it.

LinkedIn hooks.

- "A group chief executive told us her unit leads were the problem. She also told us she would not overrule them. Both were true, and the second one reshaped everything we proposed."
- "We stopped writing one proposal and started writing one per decision maker. That is when the work got approved."
- "An autonomy clause is not diplomacy. It is what makes a proposal readable by the person whose job it appears to threaten."

Cleared to publish. The decision rights mapping method. The autonomy note principle. The two part fee finding, expressed generically as a pricing structure argument with no numbers.

Never publish. The client, any business unit, any named individual, any venture, any fee. The internal context about the chief executive's frustration is internal only and must never appear in any form that could be traced.

4.5 A board of eight, split four to four

Board governance framework. Specialist hospital, Lagos. Two founding families, both founders deceased. Eight directors. Day to day operations run by a third party operating partner.

The brief we were given

Set up a board operating model that both families would accept, and do it before the next quarterly board meeting.

What we found

The board was split four to four between the two families, and one family had convened it and nominated all of the non executive directors. That family had filled a vacuum rather than staged a takeover, but the other family perceived the board as tilted and the non executives as one family's picks. The same family also held the medical directorship, which concentrated power further.

Chair selection was the actual bone of contention, and everything else was downstream of it.

The method

Legitimacy through fair process rather than through arithmetic. You cannot solve a four to four split by counting. You solve it by making the process for choosing visibly fair to both sides, which is a well established governance principle and it is what we applied.

The design that resulted:

- An **interim** non family chair, a senior finance figure trusted by both sides, legitimised by a fair process rather than by appointment, and chairing no committee so that the chair holds no operational power base.
- A **lead independent director**, drawn from the clinical and academic side and trusted by both families.
- **Five committees rather than the three a lean design would recommend.** This was a deliberate choice against best practice. More committee chairs means more standing to distribute across the two families, and standing was the scarce resource in this situation.
- The **remuneration committee chaired by the family that does not hold the medical directorship**, as a structural counterweight.
- The committee covering finance, investment and operations **co chaired across both families**, with deadlocks escalating to the full board and no casting vote.
- **Family dignity met through committee seats and a family council, never through operational roles.** This is the part that most governance advice gets wrong. The need is for relevance and respect, and giving somebody a job they cannot do in order to meet that need damages the company and does not meet it anyway.

The delivery was in two workstreams: eleven board and committee instruments, built with the company secretary, covering the board pack, agenda, board paper template, committee report, exception report, dashboard, risk summary, minutes, action log, escalation thresholds, registers and a rolling twelve month calendar. And a capability programme with a handbook, taught in three cohorts, with company law delivered by two practising lawyers while we carried the operating model and the craft. **We set the learning objectives and the lawyers owned the black letter law and confirmed the citations.**

The programme closed with a mock board meeting containing a planted conflict of interest, a planted reserved matter and a planted variance, plus a chairing rubric and a per cohort competency check.

The transferable principle

When a board is deadlocked by ownership, the currency is standing, not votes. Design the structure so that both sides have visible, real and non operational standing, and the deadlock stops being the organising fact of every meeting.

Publication guidance

Article angle. "Family hospitals and the four to four board." There is almost nothing written about family owned hospital governance in Nigeria and this is a substantial gap in the market's literature.

LinkedIn hooks.

- "A board split four to four cannot be solved by counting. It is solved by making the process for choosing visibly fair to both sides."

- "We kept five committees where best practice says three. More chairs meant more standing to distribute, and standing was the scarce resource."
- "Family dignity is met through committee seats and a family council. It is never met by inventing an operational role for somebody."
- "The chair of a contested board should chair no committee. A chair with a power base is not a neutral chair."

Cleared to publish. The fair process principle. The standing versus votes insight. The five committee reasoning. The dignity through governance rather than operations point. The instruments list. The mock meeting with planted tests as a training method.

Never publish. The hospital, either family, any director by name or description, the operating partner, and the fact that a director withdrew on a conflict. The political detail in this case is the most sensitive material in this handbook. Treat the governance principles as generic and detach them entirely from this engagement.

4.6 The phase three decision that had to be taken in phase one

Strategic options note and advisory. Conversion of four to five built residential units into a health facility, Abuja. Promoter is a practising consultant physician. Cancer care, day case surgery and family medicine.

The brief we were given

A promoter converting residential units into a facility, with a three phase plan: day case first, inpatient at half capacity second, centre of excellence third. A revenue target had been set for two years out.

What we found

The site alone does not reach the target, and the fix belongs in phase one. Modelled month by month, the site reaches roughly ninety five per cent of the target at month twenty four and does not cross it until month twenty eight. A small leased diagnostic and clinic spoke opened two months before the deadline crosses it on time and is worth more in net present value than it costs in capital.

So the phase three ambition contained a phase one decision. If the second site is not committed early, the target is missed regardless of how well the first site performs. That is the entire finding and it is the kind of thing a phased plan is structurally bad at surfacing, because phases invite you to think about each one in sequence.

Two things break this project and neither of them is demand. Construction cost overrun and schedule delay, stacked together, and then landing in a thinner market than assumed. There is headroom for about a quarter of cost overrun, or for eight months of delay, but not for both. Oncology volumes can miss by more than half and the project survives.

Working capital at month twenty four is substantial and almost entirely receivables. That is the number promoters never plan for, and it is the most common way a well conceived facility runs out of money while trading profitably.

The positioning was corrected during the engagement and the correction matters. The first draft implied that no private operator does cancer care end to end in that city. That is not true. A major internationally backed centre of excellence is under twenty minutes away and covers the whole pathway including radiotherapy. So the position is not competition, it is **underneath**. That flagship is priced against international destinations, which means the market is everybody who cannot reach those prices. Price at roughly half the flagship tariff, refer complexity upward, take the follow up and palliative work back, and never frame the new facility as a competitor.

There is a second gap, and it is the one the promoter is uniquely equipped to own. A quaternary centre is not a front door. Screening, family medicine, survivorship and home palliative care are the ends a practising family physician owns by training and that the flagship does not want.

The method

This was a prospect, not a client, so the deliverable was an options note rather than a feasibility study. Six pages. What this is and is not, said in the first paragraph. Six short market observations that demonstrate command of the sector. Three or four genuinely different businesses that could occupy the same site, presented as alternatives rather than as phases of one plan, each with capital and revenue as **ranges**, plus what has to be true and what kills it. Then a two column "if this, then expect that" table, which is the spine of the document. Then a section called what I do not know, listing the questions that would change the answers and how each would be answered.

The full financial model sat behind the note so that every range was grounded, and none of it was published.

Why. At that stage nothing has been measured and no market verified, so a confident twenty six page plan is both dishonest and the wrong sales instrument. It answers every question and leaves nobody a reason to call. A short note that shows the firm knows the sector cold, lays out the options and is explicit about what it cannot yet know is what makes the call happen.

The regulatory long poles were named specifically rather than generically, because naming them is what proves sector command: change of use consent from residential, radiation authorisation lodged at design stage rather than after construction, and the controlled drug licence for opioids lodged months before palliative services open.

The transferable principle

A phased plan hides the decisions that have to be taken early. Model the whole arc monthly, then ask which phase three commitments have a phase one deadline.

And commercially: **for a prospect, postulate possibilities, do not answer everything.**

Publication guidance

Article angle. "Phase three, decided in phase one." A short, sharp piece about why phasing hides decisions.

LinkedIn hooks.

- "Her phase three ambition contained a phase one decision. If the second site is not committed now, the target is missed no matter how well the first one performs."
- "Two things break a hospital build and neither is demand. Cost overrun and delay, stacked. There is room for one, not both."
- "The number promoters never plan for is working capital at month twenty four, and almost all of it is receivables."
- "You do not compete with the flagship. You sit underneath it and take the work it does not want."

Cleared to publish. The phasing insight. The cost and schedule headroom principle. The working capital warning. The positioning underneath a flagship argument. The options note structure, which is genuinely useful content for anybody who commissions consultants.

Never publish. The promoter, her employer, the site, the target figure, any capital or revenue number, and any reference to the nearby centre of excellence in a way that pairs it with our client.

4.7 Counting providers when the number that mattered was coverage

Provider network growth strategy. Health plan operating on cooperative insurance principles, historically northern Nigeria, opening a Lagos office to drive southern growth.

The brief we were given

Onboard around three hundred Lagos providers in one quarter, up from a baseline of about a hundred and twenty. Existing tactics, a nationwide email to three thousand providers, callbacks and contracted business development managers, were all converting poorly.

What we found

The target was the wrong number. Provider growth should be derived from where enrollees actually are, not pursued as a standalone volume count. Hitting three hundred by signing cheap and empty providers produces a longer network that is no more adequate. The metric we recommended was covered enrollee percentage and network adequacy in priority clusters.

The seed enrollee base was already in the business and nobody had looked at it. The company had northern corporate accounts, and several of those corporates had Lagos branches. Geocode those branch addresses and that is the initial Lagos enrollee heatmap. Anchor provider acquisition there first. This costs nothing and it was sitting in their own client list.

Their own brand promise was the strongest provider acquisition lever they had, and it was not being used on providers. They promise zero rejected eligible claims. Providers decline health plans overwhelmingly because of denied and delayed claims. That promise was being marketed to enrollees and never to the people whose signature they actually needed.

The affinity network was a conversion moat that email cannot touch. The company operates on cooperative insurance principles with a faith aligned identity. Warm introductions through the relevant associations, anchor providers in faith aligned facilities, and employers in the aligned finance sector as corporate accounts, are a completely different acquisition motion from cold outreach. It also means the enrollee and provider geography may skew to corridors that are not the default commercial axis everybody else targets.

And the sequencing point. The company is building its own outpatient clinics. So the network should be sequenced against that planned rollout: do not duplicate future clinic locations, and prioritise the tiers the clinics will not cover. We framed this forward looking rather than as a build versus buy argument about today.

The method

Two deep research streams run in parallel before any recommendation: the Lagos health plan market, provider landscape, adequacy standards, competitors and onboarding levers; and the affinity and existing client branch seeding thesis.

A clinician led network build as the differentiator. Rather than cold onboarding facilities, seed the network by engaging clinicians directly. Senior Lagos specialists practise across multiple hospitals, so one relationship extends member access to several facilities at once. The hospital still wins, because it earns on the bed, the theatre, nursing, diagnostics and pharmacy when a specialist brings an insured member in, which makes hospitals easier to sign rather than harder.

Two deliverables, deliberately different. A client facing proposal that holds the intellectual property back and offers a phased engagement, and an internal full strategy and execution playbook that contains everything.

The transferable principle

When a client gives you a target, check whether it is the right number before you help them hit it. A network of three hundred providers that does not cover where the members live is worse than a network of a hundred and eighty that does, because it costs more to administer and it still generates complaints.

Publication guidance

Article angle. "Network adequacy is not network size." Strong, specific and genuinely useful to every health plan executive in the country.

LinkedIn hooks.

- "A health plan asked us to help sign three hundred providers. We told them the count was the wrong number."
- "Their strongest provider acquisition argument was their own enrollee brand promise, and they had never once used it on a provider."
- "The Lagos enrollee heatmap was already inside their own client list. It was the branch addresses of their northern corporate accounts."
- "One specialist relationship can extend member access to several hospitals at once, because senior consultants practise across multiple sites."

Cleared to publish. Adequacy versus count. The claims promise insight, expressed generically. The clinician led network build, which is a genuinely novel argument in this market. The existing client branch seeding idea as a principle.

Never publish. The client, its faith alignment, its markets, its clinic plans, the target number paired with the sector, and anything about its current conversion performance.

4.8 The venture that had its sequence backwards

Group holding company structuring and healthcare venture design. Family group, Nigeria, spanning education, energy, agriculture, healthcare and retail. The healthcare venture covers eldercare.

The brief we were given

Structure a technology first holding company over several ventures, and design the healthcare venture: home care services and residential eldercare, with home care as the wedge and residential in a later phase.

What we found, eight weeks later

The sequence was inverted, and inverting it back changed the critical path. The business is a residential eldercare business. Home care and virtual consultations are not the business with a care home added later. They are strategic positions that fund the build, prove the care standard, establish the brand and produce the admissions pipeline that fills the beds on opening day.

That reframing strengthened the diaspora thesis rather than weakening it. A daughter in London cannot supervise a visiting carer. The diaspora has already accepted that somebody else provides the care, which makes it the segment least troubled by the cultural objection to a care home.

The cap table then broke, and it broke in a way that was nobody's fault and would have been very hard to unwind later. Once the core of the venture is a building, a single company cap table means the operating partner's stake is a stake in real estate the family funded. That is neither what the operator earned nor what the family intended to give.

So: **separate the building from the business.** A property company under the family holding company owns or leases the site. The operating company carries the agreed shareholding, holds the brand, the platform, the clinical licences and the staff, and pays arm's length rent. This also ring fences clinical liability away from the asset, allows the property to be financed as property, makes the second home a repeat rather than a renegotiation, and stops one of the two assets always being valued on the wrong multiple.

This was urgent because counsel had already been instructed and was drafting against the single entity assumption.

And the finding that has nothing to do with structure. The waiting list had never been opened, and eight weeks had been lost. For a residential business, **the waiting list is the occupancy plan.** It is not a mailing

list and it is not a marketing nicety. It is the thing that determines whether the home opens at forty per cent occupancy or eighty.

The method

Build the demand capture before the entity exists. The front door went live within days of the reframing: a public page and a waiting list form, carrying the venture's own brand with no trace of the consultancy on it, because a family arriving has no reason to have heard of us.

Capture segmentation, not contact details. The form captures whether the enquirer is based outside Nigeria, which country, which part of the city care is needed in, the relationship to the person needing care, whether they want residential or home care or both, the care needs, and the urgency. Those fields are the occupancy plan. The headline numbers on the internal view are total, diaspora share, share who want a place, share who are urgent, and share who came by referral, sorted by urgency.

Say what is true. The page states plainly that the home is not open yet and that it will not open until it is registered and inspected. No prices, because the earlier pricing ladder was illustrative. The clinical lead is referred to by role and not named, because his terms are not papered and a licensing question about him is open.

Data protection handled properly and early. Consent text stored verbatim on the record with a timestamp, the enquirer's address hashed rather than stored, and registration with the data protection regulator before the first family record exists.

Build residential ready from the first line of code. Model the client rather than the service. Put occupancy in the data model now, even though the screens ship later. And carry escalation signals on every home care client, acuity trend, carer strain, falls, hospital admissions, and whether the decision maker is abroad, because that dataset is the admissions pipeline.

The transferable principle

Ask what the business actually is before you phase it. A wedge that funds and fills the core is a completely different thing from a first phase that grows into the core, even though they look identical on a timeline.

And: **when the core becomes an asset, the cap table has to split.** Equity earned for building and running a business should not silently become equity in somebody else's property.

Publication guidance

Article angle. "Eldercare, the diaspora and the occupancy problem." Or, more technically, "PropCo and OpCo: why a care venture cannot sit in one company."

LinkedIn hooks.

- "The home care business was not the business. It was the bridge that funds the build and fills the beds."
- "A daughter in London cannot supervise a visiting carer. That single fact is why the diaspora is the least resistant segment for residential eldercare."
- "For a residential business, the waiting list is not a mailing list. It is the occupancy plan."
- "Once the core of your venture is a building, a single company cap table gives your operating partner a stake in your property. Split it before counsel starts drafting."

Cleared to publish. The sequencing insight. The diaspora argument. The waiting list as occupancy plan. The PropCo and OpCo argument in full, which is broadly applicable and genuinely useful. The segmentation approach. The data protection discipline.

Never publish. The family, the group, any sector it operates in, the venture's brand name, the clinical lead, the shareholding percentages, and anything about the open licensing question.

4.9 Auditing our own instrument

Internal. Product diagnostic on the firm's own leadership assessment platform.

What we did

We ran a diagnostic against every completed assessment session on our own platform and published the findings internally without softening them.

Four things came back.

The emotional intelligence scale was not discriminating. Scores were collapsed at the ceiling, with a median at ninety five and a floor at eighty across every sub scale. An instrument where almost everybody scores very highly is measuring social desirability, not emotional skill. The likely cause is option weights mapped so that most non pathological choices score at the top of a five point scale, combined with high social desirability on the item content and no validity indicators to detect inflation.

The 360 funnel collapsed. Almost every subject created a 360 request. Seventy two per cent of those requests never had a single rater invite sent. Two separate data paths existed for 360 responses and they were not connected to each other.

There was no integrity construct in the instrument at all. The modules collectively measure behavioural style, values, emotional skill, career stage identity and culture. None of them measures integrity. Adding it is net new construct work, not an upgrade of an existing scale.

Response time was not captured on any item. That removes the cheapest available diagnostic for careless responding.

Why this belongs in a marketing handbook

Two reasons.

The first is a constraint. Until these are fixed, **we do not claim predictive validity, normative percentiles or integrity measurement for the assessment.** Not in a deck, not in a post, not in a proposal. The methodology in section 2.4 is the direction of travel and it is real, but the current instrument does not yet support every claim that methodology would justify.

The second is an opportunity, and it is a real one. **Publishing a candid audit of your own product is the most credible thing an assessment company can do**, because the entire category is built on claims that buyers cannot independently verify. A piece written in the form "here is what we found when we audited our own instrument, and here is what we are changing" would do more for Maarova's credibility with a sophisticated buyer than any number of feature posts. It needs Debo's sign off and it should be written carefully, but it is the single highest value piece of content available to that brand.

Publication guidance

Article angle. "We audited our own assessment. Here is what was wrong with it." Very high value, needs explicit clearance.

LinkedIn hooks.

- "An assessment where almost everybody scores ninety five is measuring social desirability, not emotional intelligence."
- "Seventy two per cent of our 360 requests never had a single rater invited. The instrument was fine. The funnel was not."

- "Most assessment companies will never tell you what is wrong with their instrument. Here is what was wrong with ours."

Cleared to publish. Nothing yet. This section is internal until Debo clears the piece.

4.10 A policy brief on the housemanship bottleneck

Policy work, unpaid, written for a healthcare industry federation to put on its own letterhead and send to a federal minister.

What it was

The president of a healthcare industry federation asked publicly for somebody to write a paper proposing solutions to the medical internship placement bottleneck. Debo offered, and wrote a policy brief rather than an opinion piece, in the federation's collective voice so that its leadership could put it on their letterhead and send it to the minister.

The argument

Bribery and the pain of centralised posting are symptoms. The real problem is structural, and it separates cleanly into four root causes, each with a matching remedy.

Supply is too small and too narrow, with private hospitals held back by a high accreditation bar and too few slots. Demand is rising by policy. Funding is fragile and not portable, so a place cannot follow a graduate. And allocation is opaque and gameable.

The key figure: around two thousand of roughly six thousand annual graduates are stranded, against accredited capacity of about four thousand. And the door is open, because the regulator has already told the Senate that the fix involves state and private hospitals.

Why it matters to marketing

This is the model for our policy work and it is a category most consultancies in this market do badly.

It carries no consultancy branding. The brief is neutral, written for the federation, with our involvement reduced to a signature line. Debo is a member of that federation and no longer in its leadership, and his diaspora network role is kept deliberately subtle so as not to overshadow the body whose voice the document carries.

It is rigorously sourced. Every figure is attributable to the regulator's registrar through Senate testimony and national press. That is what allows a federation to put it on letterhead and a minister's office to read it without discounting it.

The influence is the return, not the attribution. Work like this builds the relationships that produce mandates. It does not produce them directly and it should never be written as though it might.

Publication guidance

Article angle. The four cause structure of the housemanship bottleneck is publishable in its own right under our own name as sector commentary, using the public figures, provided it does not present itself as the federation's brief or imply endorsement.

LinkedIn hooks.

- "Two thousand Nigerian medical graduates a year cannot start work. Accredited capacity is four thousand. Graduating cohorts are six."
- "One hundred and fourteen hospitals are accredited for housemanship. Forty four are on the central portal. Seventy are not."
- "The bribe is a symptom. Four structural causes sit underneath it, and each has a matching remedy."

Cleared to publish. The public figures, all attributable. The four cause structure as our own analysis.

Clear with Debo. Any reference to the federation, the brief, the minister or Debo's authorship of it.

Part Five: The method library

These are the reusable things the firm does. Each one has a name, a shape and at least one engagement behind it. They are the raw material when you need an angle and the case studies are exhausted, and several deserve to be written up as standalone method pieces.

5.1 The four stream diagnostic

Never rely on one kind of evidence. Interviews, an anonymous staff instrument, a patient or customer instrument, and the client's own documents. Four streams chosen so that no two fail the same way. Interviews capture what people believe. Surveys capture what people will not say to your face. Documents capture what actually happened. When streams disagree, the disagreement is itself the finding.

5.2 The ten case trace

Pick ten real cases on the morning, with the client present so nothing is curated. Follow each through every link in the chain from first contact to final outcome and payment. Capture the same numbers on each. Ten cases traced completely beats a hundred sampled partially, because every gap you find is specific rather than theoretical.

5.3 The forced tradeoff leadership survey

For founder and board alignment. Not a Likert questionnaire, because everybody agrees with everything on a Likert scale. Instead: constant sum allocations where a hundred points must be split between competing priorities; paired forced choices with no midpoint; and second order questions asking what the respondent thinks the others will say.

The second order questions are the whole point. They produce the **perception gap**, which is where a group's real problem usually is. In one engagement, every founder chose to invest in staff, and half of them assumed their colleagues would protect margin instead. They were aligned and did not know it, which meant nobody was arguing for the thing they all wanted.

Results are presented as a completion tracker naming who is outstanding, each item tagged agreed or split, the perception gap tagged where a group misread itself, and open text attributed by name.

5.4 Measured geometry

Where CAD drawings exist, recover the real dimensions rather than accepting the brief. Areas are measured, not assumed. Applies well beyond buildings: the general form is that the client's summary of an asset is a document about the client, and the underlying file is a document about the asset.

5.5 Decisions with options, not recommendations

Boards act on choices and defend against instructions. Present three to five decisions, each with genuinely different options, each option scored on the same axes: speed to cash, capital required, management strain, and whether it actually relieves the binding constraint. Say which you would back and why, with the caveat attached.

5.6 The options note

The deliverable for a prospect who has not yet engaged the firm. Six pages. What this is and is not, in the first paragraph. Market observations that demonstrate command. Three or four genuinely different businesses on the same asset, with ranges rather than point estimates, and what has to be true and what kills each. An "if this, then expect that" table as the spine. A section on what we do not know. One page on how we work with no fee tables. The model sits behind it and is not published.

5.7 Standard, concession, net

Show the full standard fee, one named concession line, and the net. Full transparency about what has been given up, exactly once, rather than a discounted number presented as the price.

5.8 Give to get

The data product pattern. A visitor gets full value only after contributing. Ratings are always visible, review text unlocks on contributing a review. The salary map unlocks on completing a salary survey. It solves cold start and it produces the dataset that is the actual asset.

5.9 One link, not six attachments

Everything a client team needs behind a single URL: the documents, the information requests, the uploader, the surveys, the print pack and what happens next. Internal material stays off it, because the link goes to the whole team.

5.10 Verify the destination before you send

Post the real payload to the live endpoint, read the row back, assert that every field survived, then delete the test rows. A live page can sit in front of a dead endpoint, and a 200 response can sit in front of a parser that read nothing.

5.11 Paper and web cannot drift

Where an instrument exists as both a web form and a printed sheet, generate both from one definition. Two hand maintained copies of the same questionnaire will diverge, and then the data cannot be combined.

5.12 Check every claim of absence

Before writing that something does not exist, confirm that it was actually requested and not supplied. Inferring absence from a missing document is the most common way a diagnostic loses credibility, and one wrong finding discounts every correct one around it.

Part Six: Your first ninety days

Weeks one and two: learn and audit

- Read this handbook end to end. Ask about anything that does not make sense.
- Read the three brand guide PDFs in `docs/`: CFA, Maarova and CadreHealth.
- Read `docs/haven-cover-email-kabir.md` for Debo's actual voice before you draft anything in his name.
- Read the social media strategy at `docs/social-media-strategy.md`, which predates you and needs updating rather than replacing.
- Audit what exists: every social account, whether it is claimed, what is posted, what the follower base actually is.
- Audit the website: what pages exist, what is stale, what the insights section contains.
- List every brand asset that exists and every one that is missing, against section 2.6.
- Sit in on one live engagement conversation if Debo will let you. Nothing substitutes for hearing how a client actually talks.

Weeks three and four: fix the foundations

- Close the brand hygiene items you can close without a designer. Claim the handles. Set up the accounts that do not exist.
- Commission the CadreHealth logomark. It is the highest priority open brand task in the firm.

- Resolve the credential claim inconsistency in section 2.6, with Debo.
- Build the templates the firm does not have: case study, data insight, quote card, salary comparison, team spotlight.
- Set up tracking. UTM conventions on every link, so that an inbound enquiry can be traced to the thing that produced it.
- Draft the approved boilerplate in section 7.2 into a single place everybody can copy from.

Weeks five to eight: publish

- Turn three case studies from Part Four into insights articles. Start with 4.2, 4.3 and 4.6, because they are the least sensitive and the most distinctive.
- Take each article through the clearance checklist in section 7.3 and get Debo's sign off before anything goes live.
- Build the LinkedIn cadence for CFA. Four a week, drawn from the hooks in Part Four.
- Start the CadreHealth content engine on X and Instagram. Salary content first, because it performs and because it feeds the give to get loop.
- Do not run any bulk email until the CadreHealth suppression back fill is done. Confirm this with the engineering side before you plan a campaign around a list.

Weeks nine to twelve: compound

- Get two more named client references cleared. Ask at the point an engagement is going well, not months afterwards.
- Write the Maarova methodology piece. It is the strongest unpublished asset in the firm.
- Propose the annual workforce report. It is the press, backlink and authority play for CadreHealth and it only works if it is started early enough to gather data.
- Bring a proposal to Debo for the three campaigns he wants running before the end of the year.

What to ask for when you need it

- Anything about an engagement, a client, a figure, or whether something can be published: Debo.
- Anything about how an audit actually ran, or the product roadmap: Tito.
- Anything about pipeline, payers or corporate relationships: Dorothy.

Part Seven: Appendices

7.1 Glossary

If a piece of copy uses one of these without explaining it, check that the reader will know it. Our internal audience does. A development finance reader may not.

Term or acronym	What it means
MDCN	Medical and Dental Council of Nigeria. Registers doctors and dentists.
NMCN	Nursing and Midwifery Council of Nigeria.
PCN, MLSCN, RRBN	Regulators for pharmacy, medical laboratory science, and radiography.
HEFAMAA	Lagos State's health facility monitoring and accreditation agency. Registration is a state requirement.
NHIA	National Health Insurance Authority. Successor to the older NHIS.

Term or acronym	What it means
HMO	Health maintenance organisation. Nigeria's dominant private health plan structure.
Takaful	Cooperative insurance structured on Islamic principles.
CONMESS, CONHESS	Public sector salary structures for medical and health workers. High search volume.
NARD	National Association of Resident Doctors. Strike action is a recurring news driver.
NMA	Nigerian Medical Association.
Housemanship	The one year internship a Nigerian medical graduate must complete before full registration.
Japa	Colloquial Nigerian term for emigrating. Use with care in formal copy.
Locum	Temporary clinical cover, paid by shift or session.
Secondment	Placing our own staff inside a client organisation.
Fractional leadership	A senior operator working part time in a defined executive role.
NICU	Neonatal intensive care unit.
CSSD	Central sterile services department. Where instruments are sterilised.
Tariff	A facility's price list, line by line.
Establishment	The staffing a service is actually resourced with, as distinct from the organisation chart.
Receivables	Money billed and not yet collected. In Nigerian private healthcare, mostly owed by health plans.
VMI	Vendor managed inventory. The supplier owns the stock until it is used.
EBITDA	Earnings before interest, tax, depreciation and amortisation. The standard operating profit measure.
Capex and opex	Capital expenditure and operating expenditure.
PropCo and OpCo	A property company and an operating company, held separately.
SPV	Special purpose vehicle. A company formed for one venture.
NDPA	Nigeria Data Protection Act 2023. It supersedes the older NDPR, which is still widely and wrongly cited.
DFC	Doctors Foundation for Care. Debo's diaspora physician network, over five hundred members.
ICF	International Coaching Federation. The credentialing body our coach network draws on.

7.2 Approved boilerplate

One line.

Consult for Africa is a healthcare management consulting firm working across Africa in hospital turnaround, clinical governance, strategy and operational excellence.

Short, fifty words.

Consult for Africa is a healthcare management consulting firm operating from Lagos and Abuja. We turn around struggling hospitals, install clinical governance, and stand up fractional leadership for African health systems. Built by clinicians and operators who know the system from the inside, we do not hand you a report and walk away.

Long, one hundred and twenty words.

Consult for Africa is a healthcare management consulting firm founded in 2024, operating from Lagos and Abuja across Nigeria and the wider continent. We work with hospital owners, health system executives, boards, investors and governments on hospital turnaround, clinical governance, strategy and growth, digital health, health systems strengthening and fractional leadership.

Across the continent, the gap between a struggling hospital and a thriving one is rarely the building or the equipment. It is execution, leadership and people. Consult for Africa exists to close that gap. Built by clinicians and operators who have run health institutions from the inside, we do not hand you a report and walk away. We build, staff, and stand behind the outcome.

CadreHealth, short.

CadreHealth is the career platform for Nigerian healthcare professionals. Real salary data, honest hospital reviews, verified credentials, and a readiness score calibrated to UK, US, Canada and Gulf markets. For doctors, nurses, pharmacists and every cadre in between.

Maarova, short.

Maarova is a psychometric assessment and coaching platform for healthcare leaders, calibrated to the African context rather than translated into it. Six dimensions, certified African coaches, and a measurable arc from assessment to retention.

Mezo Health, short.

Mezo Health is a verified specialist network and hospital operating stack. Consult on our network of facilities or by teleconsultation while we fill your clinic, run your schedule and collect your payment. Your own practice, without building one.

Debo's bio, short.

Dr Debo Odulana is Founding Partner of Consult for Africa. A physician trained at the University of Sharjah with a masters in international health management from Imperial College Business School, he has served as Chief Executive of Cedarcrest Hospitals in Abuja and as Chief Innovation and Strategy Officer at Evercare Hospital Lekki. He founded Doctoora, which he built to twenty one states and sold to Evercare. He is President of the Doctors Foundation for Care.

7.3 The clearance checklist

Run every external piece through this before it goes out. All of it, every time.

- No em dash anywhere in the piece.
- No "AI powered", "AI driven" or equivalent.
- No client named, and no combination of details that identifies a client.
- No absolute client financial figure unless specifically cleared.
- No clinical incident, patient outcome, mortality or regulatory finding.
- No named individual other than our own people, and only with their agreement.
- No internal platform named inside a client deliverable.

- No price, and no defence of a price nobody has challenged.
- Every external statistic attributable to a public source, and the source noted.
- Anything in Debo's name reads like him: long connected sentences, no contractions, first person singular for judgement, signed Dr Debo Odulana.
- Nothing puts a question or a brief in a client's mouth that they did not ask.
- Would the client be pleased to read this.
- Debo has seen it.

7.4 Where things live

What	Where
Client deliverables	`docs/` in the consult-for-africa repository, as PDFs
Document build scripts	`scripts/build-*.py`
Brand guides	`docs/brand-guide-cfa.pdf`, `-maarova.pdf`, `-cadrehealth.pdf`
Logo assets	`docs/c4a-logo.png`, `docs/c4a-logo-reversed.png` for dark backgrounds, `docs/c4a-icon.png`
Social media strategy	`docs/social-media-strategy.md`
Debo's voice reference	`docs/haven-cover-email-kabir.md`
This handbook	`docs/marketing/cfa-marketing-handbook.md`, built by `scripts/build-marketing-handbook.py`

A note on deliverables. Every client facing document this firm produces is a PDF, built by a script, output to `docs/`. Not a web page, not a hosted link. Our readers are hospital promoters, boards and family offices who need something they can email, print, take into a board meeting and read on a phone with no login and no dependency on a platform staying up. Marketing content is different and lives on the web. Client documents are not.

7.5 The three things to remember

One. Our proof is the method. We can show exactly how we work without ever naming who for, and almost nobody else in this market can, because almost nobody else works this way.

Two. Our audience is senior, clinical and sceptical. Write to them as peers. Specificity earns their attention and adjectives lose it.

Three. When you are unsure whether something can be said, ask. A softened version of a sharp finding is usually worse than either the finding or the silence.